

BODY RECODE · FIELD GUIDE

The Insulin Tells

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Body Recode · Performance Coaching

The Insulin Tells

The full set. Timing, storage, and the four that rule the other patterns out.

The post gave you four. Here is the whole list, plus the part that actually matters, which is how you tell insulin apart from the three drivers that look like it.

Insulin is the most over-diagnosed pattern of the four and the most under-diagnosed at the same time. Over-diagnosed because everyone with a soft middle gets told it is carbs. Under-diagnosed because the real signature sits on the **back**, and nobody checks the back.

One rule before the list. **No single tell means anything.** These cluster or they are noise. Three or four of them lining up is a read worth acting on. One of them on its own is a Tuesday.

The timing tells

Insulin is a fuel-handling pattern, so its clearest signals are tied to the clock, not the mirror.

- 1. The afternoon dip, roughly 2 to 4pm.** The one that has nothing to do with how you slept. You can sleep eight hours and still lose the middle of the afternoon.
 - 2. Heavy and foggy for about an hour after eating.** Worst after lunch. Not full, not tired. Slowed.
 - 3. Cravings arrive hardest after dinner.** The day is done, the discipline is gone, and the craving does not feel like a choice.
 - 4. Hungry again soon after a proper meal.** As though it did not register.
 - 5. You sometimes feel sharper on a day you ate less than on a day you ate properly.** That is a genuinely odd thing to notice about yourself, and it is one of the more specific tells here.
 - 6. The same food lands differently depending on when you eat it and what is next to it.** Rice at lunch with protein and vegetables is fine. The same rice on its own at 9pm is not.
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The storage tells

7. It sits at the back. Mid-back, lower back, the flanks, the sides. The front of the stomach stays comparatively flat.

This is the discriminator most people miss, and it is the one worth learning. Everyone checks the front. The mid-back skinfold is one of the strongest single predictors of insulin resistance there is, in some studies outperforming a scan for visceral fat.

8. The waist is firm and full rather than soft. The deep layer is what tracks with insulin.

And the correction that goes with it: *softening evenly all over is not an insulin sign.* The superficial layer, the one that reads as general softness, has no insulin association at all. If your honest answer is “it is fairly even, I could not pick one spot”, that does not type as insulin, and it does not type as much of anything on storage alone.

The response tells

9. Cutting carbs hard works for about a week, then stalls. And it comes back when you stop.

10. A walk after a meal changes how you feel out of all proportion to the effort. Ten or fifteen minutes should not make that much difference. On this pattern it does.

11. The cravings feel physical rather than mental. Not a discipline problem. A signal.

One that belongs with your GP, not with a checklist

Skin tags, or darker velvety patches in the folds of the neck, armpits or groin, can travel with this pattern. **That is a conversation with your doctor, not something to self-diagnose from a one-page guide, and it is not something to act on from here.** Mentioned because it is real and because you should know to raise it, not so you can score yourself on it.

The four that rule the other patterns out

This is the part the checklists leave out, and it is the whole reason people run the wrong correction for a year. **Three of the four hormonal drivers push fat toward the middle.** So a central pattern narrows nothing on its own.

IF YOU ALSO SEE	IT IS POINTING AWAY FROM INSULIN, TOWARD
The middle filling while your arms and legs get thinner	Cortisol. Limb thinning is the cortisol signature, and it is the one thing insulin does not do.
Storage at the hips, glutes and outer thighs , or that pattern starting to move central, in a woman whose cycle has changed	Oestrogen. Cycle status and direction of travel decide this one, not the location.
Muscle, tone and drive falling together while the middle fills, in a man	Testosterone. The giveaway is the muscle going, not the fat arriving.
Nothing much lining up cleanly at all	Possibly none of them, which is a real answer. A body that is responding well does not type.

Front versus back is what cleanly separates insulin from cortisol. It is the single most useful distinction on the whole map, and it takes about four seconds to check.

What this one answers to

Insulin is the one pattern where eating less and moving more is genuinely fair. Reduce the load and it responds, which is why it is the pattern most likely to make a generic plan look like it worked.

But the lever is **timing and pairing before restriction**. What sits alongside the carbohydrate, and when in the day it arrives, moves this more reliably than cutting it out does. Elimination works for a fortnight and then you are living a smaller life for the same result.

Two honest warnings

This narrows the field. It does not hand you a verdict. A pattern is a read, and a read can change as the body changes. Anyone giving you a permanent label off a checklist is selling something.

If three or four of these are yours, that is worth acting on. If one is, it is not. Forcing yourself into the closest-looking box is exactly how people end up running a correction aimed at a driver they do not have.

Want the actual read rather than a best guess?

The free 14-day Body Decode Challenge takes an intake on day zero, scores eight markers against your own baseline on day seven, and gives you the full pattern read on day fourteen. Which of the four is running yours, and what that specific one answers to.

Free. Nothing to pay to get the read.

bodyrecode.au/challenge